

METASTATIC ENDOMETRIAL CANCER PRESENTING AS SUSPECTED PRIMARY LUNG CANCER: A CASE STUDY

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INTRODUCTION

ENDOMETRIAL CARCINOMA IS THE SECOND MOST COMMON GYNECOLOGICAL MALIGNANCY WORLDWIDE AND A MAJOR CAUSE OF GYNECOLOGIC CANCER-RELATED MORTALITY. PULMONARY METASTASES MAY OCCASIONALLY REPRESENT THE INITIAL PRESENTATION OF ADVANCED DISEASE. WE REPORT A CASE OF METASTATIC ENDOMETRIAL CARCINOMA PRESENTING PREDOMINANTLY WITH RESPIRATOR

CASE REPORT



**55-YEAR-OLD
MULTIPAROUS WOMAN
(PARA 5+2)**



**CHRONIC
PRODUCTIVE COUGH
2 MONTHS**



**ANOREXIA &
UNINTENTIONAL
WEIGHT LOSS
8 KG**

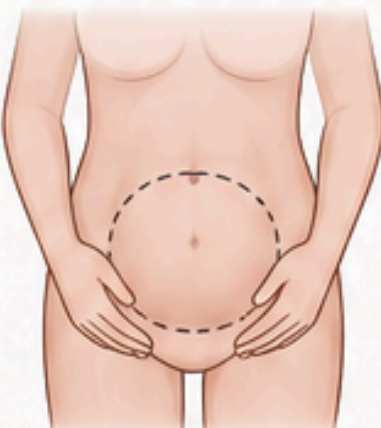


**Prolonged abnormal
uterine bleeding
1 MONTH**

1 HISTORY

- Treated by multiple GPs and traditional healers without improvement.
- Over the preceding 3 years, infrequent menstruation (~once yearly), attributed to perimenopause.
- No abdominal pain.
- No history of ectopic or molar pregnancy.

2 PHYSICAL EXAMINATION & INITIAL INVESTIGATIONS



- Suprapubic mass ~10 × 12 cm



- Respiratory examination unremarkable



Hemoglobin **11.9 g/dL**



Sputum culture **Negative**

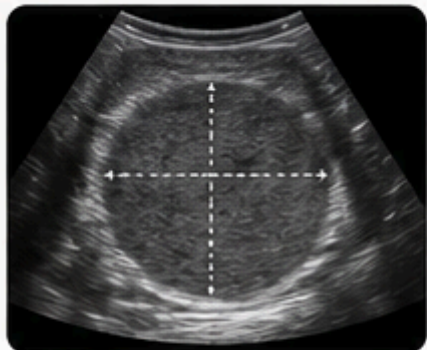


3 consecutive AFB smears **Negative**

3 DIAGNOSTIC WORKUP

TRANSABDOMINAL ULTRASOUND

Heterogeneous uterine mass measuring 10 × 12 cm



CHEST RADIOGRAPHY

Multiple well-circumscribed rounded opacities (cannonball lesions)



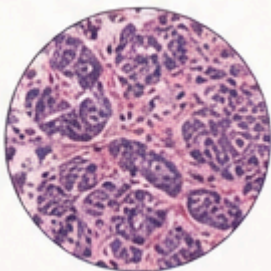
ENDOMETRIAL SAMPLING

Initial pipelle sampling: **Inconclusive**

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Repeat histopathology: **HIGH-GRADE UNDIFFERENTIATED MALIGNANT NEOPLASM**

Suggestive of undifferentiated uterine sarcoma or undifferentiated endometrial carcinoma

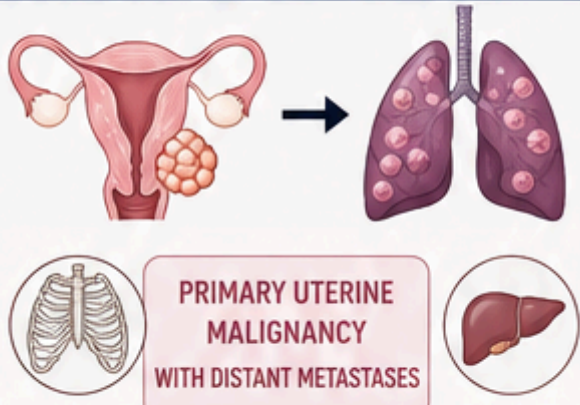


CONTRAST-ENHANCED CT SCAN

- Large lobulated uterine mass
- Multiple pulmonary nodules
- Metastatic lesion involving left 6th rib
- Suspicious hepatic lesion



4 SUMMARY OF FINDINGS



PRIMARY UTERINE MALIGNANCY WITH DISTANT METASTASES

5 MANAGEMENT



Referred for PALLIATIVE CARE

- Symptom control
- Psychosocial support
- Quality of life focus

DISCUSSION:

ENDOMETRIAL CARCINOMA COMMONLY AFFECTS POSTMENOPAUSAL WOMEN AGED 55–65 YEARS AND IS ASSOCIATED WITH PROLONGED UNOPPOSED ESTROGEN EXPOSURE (2). PULMONARY METASTASES ARE THE MOST FREQUENT SITE OF DISTANT SPREAD AND TYPICALLY PRESENT AS MULTIPLE BILATERAL PULMONARY NODULES (3,4). IN THIS CASE, RESPIRATORY SYMPTOMS SECONDARY TO PULMONARY METASTASES WERE THE PRIMARY PRESENTING COMPLAINT, RESULTING IN DELAYED RECOGNITION OF THE UNDERLYING GYNECOLOGICAL MALIGNANCY. THE ADVANCED STAGE AND AGGRESSIVE NATURE OF THE DISEASE PRECLUDED DEFINITIVE TREATMENT.

CONCLUSION:

THIS CASE HIGHLIGHTS THE AGGRESSIVE PRESENTATION OF METASTATIC ENDOMETRIAL CARCINOMA AND THE IMPORTANCE OF CONSIDERING GYNECOLOGICAL MALIGNANCY IN POSTMENOPAUSAL WOMEN PRESENTING WITH RESPIRATORY SYMPTOMS AND PULMONARY NODULES. DETAILED GYNECOLOGICAL HISTORY AND EVALUATION REMAIN ESSENTIAL FOR EARLY DIAGNOSIS AND TIMELY MANAGEMENT.

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6 KEY LEARNING POINTS



Cannonball lesions on chest imaging can be due to metastasis from extrapulmonary malignancies.



Consider gynecological malignancy in women with respiratory symptoms and abnormal uterine bleeding.



A systematic approach and tissue diagnosis are crucial for accurate diagnosis.



Early recognition allows timely referral and appropriate palliative care planning.



A thorough evaluation beyond initial assumptions can reveal the true primary and guide appropriate patient-centered care.